



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : نهاد على محمد منصور
المحافظة : القاهرة الحي / المدينة : النزهه
رقم البطاقة : 26012013400195 رقم التليفون : 01206077956
العمر (تاريخ الميلاد) : 01-12-1960 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : أورام ليمفاويه
نتائج الأبحاث : (مرفق)

التشخيص النهائي : أورام ليمفاويه
خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي ونقل دم ومشتقاته
[13491]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: نهاد علي محمد منصور
National ID: 26012013400195



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
---:--- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

A patient of lymphoma for Supportive and palliative Treatment and blood transfusion/blood products

الوقت :

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاثمين





رسوم الاستعلام 20 جنية

تاريخ الإصدار: 2026-6-4

وهذا الاستدراك برئاسة الهيئة

إلى من يهمه الأمر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين متقدمة للتفاهع الفاشعين لقانون التأمين
الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "قوى العاملة والمعدلات
والارامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012
الأطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .

وطبقا لقاعدة البيانات المرسله من التأمينات الاجتماعية

وطبقا لقاعدة البيانات المرسله من التأمين الاجتماعي

والبحث تبين أن



السيد (ة) نهاد علي محمد بطاقة رقم 26012013400195

تليفون رقم: 01288883276

وطبقا إلى خطاب من التأمينات الاجتماعية بغير عدم الالتحاق

بغير من الكفالت غير المتكفله بالتأمين الصحي صدر هذا الخطاب دون اننى مسئولية على الهيئة
العامة للتأمين الصحي حال طسوع المواطن لقانون التأمين الصحي الشامل
وشكرا

تاريخ الطباعة: 2026-6-4

الموقف
شادي حسن العيسوي

القادة مركز نظم المعلومات
بمكتب

مستقبل 1589474



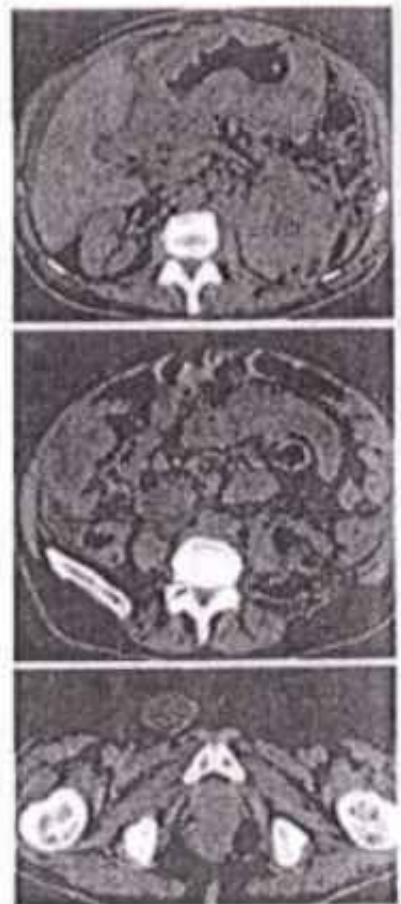
الخطاب سارو لمدة ثلاثة أشهر من تاريخ إصداره إلى حالة تغير حالة المواطن من قبل الهيئة العامة للتأمينات الاجتماعية أو
وزارة التأمين الاجتماعي أو وزارة التجارة والتجارة

الإدارة المركزية لتنظيم المعلومات والتحول الرقمي

Name: Mr. Nehad Ali Mansour
Ref.Prof.Dr.: Reda Fawzy Helmy
Date: 27/04/2026

128 CT scan of the Abdomen & Pelvis was performed without contrast with special attention to the urinary system revealed:

- Significant diffuse circumferential polypoidal heterogeneous gastric mural thickening involving mainly the gastric fundus and body and to a lesser extent the gastric antrum reaching a maximum thickness of 3.7cm with no evidence of gastric outlet obstruction.
- The left kidney is enlarged in size with diffuse parenchymal edema associated with pronounced perinephric fat smudging and nodular thickening of the surrounding Zuckerkindl's and Gerota's fascia suggesting regional metastatic infiltration.
- Mild left renal pelvis fullness with diffuse circumferential upper ureteric mural urothelial thickening yet no obstructive lesions or stones.
- The liver is enlarged in size measuring about 16.2cm showing homogenous CT parenchyma associated with diffuse periportal edema.
- The gall bladder shows average capacity with few intraluminal stones or average size 5x5mm.
- Innumerable number of enlarged malignant looking pelviabdominal lymph nodes detected mainly at the gastric, celiac, peripancreatic, portahepatis, left perinephric, bilateral para-aortic, aortocaval, retrocaval and retrocrural groups, the largest seen at the gastric region measuring about 3x3cm.
- Mild to moderate pelviabdominal hyperdense ascites.
- Diffuse small intestinal mural thickening suggesting mural edema.
- Multifocal pelvic ileo-ileal short segment intussusception reaching about 4.5cm in length with no significant proximal intestinal dilatation.
- Pronounced diffuse mesenteric fat smudging.
- Right oblique inguinal hernia with fluid content.
- The lower chest cuts show bilateral pleural effusion, minimal on the right and mild on the left with left lower lobular posterior basal subsegmental subpleural pulmonary atelectasis.
- Multiple lingular subpleural pulmonary atelectatic bands with intervening ground glass haze.



Name: Mr. Nehad Ali Mansour
Ref.Prof.Dr.: Reda Fawzy Helmy
Date: 27/04/2026

- *The urinary bladder appears intact on the basis of non enhanced study. No stones within.*
- *Spleen, pancreas and suprarenal glands are of average size, regular margins with homogeneous attenuation pattern of the parenchyma. No focal mass lesion.*

Opinion :

- *Suspicious looking diffuse gastric mural thickening suggesting infiltrative malignant neoplastic process giving CT appearance of Linitis plastica.*
- *Bulky left kidney with evidence of parenchymal infiltrates and surrounding focal metastatic deposits.*
- *Gall bladder stones.*
- *Mild to moderate ascites.*
- *Innumerable number of suspicious looking periaabdominal lymph nodes mostly metastatic in nature.*
- *Multifocal distal ileo-ileal intussusception with no definite evidence of proximal intestinal dilatation.*
- *Right oblique inguinal hernia.*
- *Bilateral pleural effusion mainly on the left side.*

Best regards
Dr. Amira Mohamed, MD

Dr.
Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis

M. Sc. MD Pathology, Asst. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : نهاد علي محمد منصور

Age : 65 Y : 5 M : 7 D

Gender : Male

Referred By Prof. Dr. : كيرلس عاطف

Clinical Diagnosis : Large malignant ulcer invading gastric antral mucosa and extending to duodenal bulb

Nature of Specimen : Block (Endoscopic biopsy, labelled as 1053620)

Receiving Date : 2020-05-08

Analysis ID : 1055769

Conf. Code : 0600

Patient ID : 771992

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain.

The sections were treated against:

- CD 20 \ CD 3 \ Ki67 \ CD 10 \ C-MYC \ Bcl 2 \ Bcl 6 \ MUM 1.

Results :

Neoplastic lymphoid cells are POSITIVE for CD 20.

Neoplastic lymphoid cells are NEGATIVE for CD 3.

90 % of neoplastic lymphoid are POSITIVE for Ki67.

Neoplastic lymphoid cells are NEGATIVE for CD10.

40% of neoplastic lymphoid cells are POSITIVE for C-MYC.

Neoplastic lymphoid cells are POSITIVE for Bcl 2.

Neoplastic lymphoid cells are POSITIVE for Bcl 6.

Neoplastic lymphoid cells are POSITIVE for MUM 1.

Conclusion :

Block, FINDINGS ARE COMPATIBLE WITH LARGE B-CELL LYMPHOMA OF POST GERMINAL CENTER ORIGIN (ACTIVATED B-CELL TYPE), TRIPLE EXPRESSOR.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

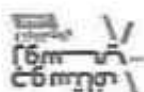
Elia Anis



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عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تيلفون : ٠١٩١٢٩٤٥١٤ - ٠١٢٨٧٠٠٥٢١٧



Nibad Ali Mohammed

2026-06-01

Prof.Dr. no doctor

Goustaive Roussey International Egypt

Patient's Name:

Date:

Referred by:

Hospital:

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH

CORONAL RECONSTRUCTION: -

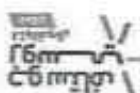
Findings:

- Bilateral moderate pleural effusion with underlying consolidation collapse.
- Bilateral upper lung lobes extensive ground glassing mounting to consolidations with mild bronchiectatic changes.
- Bilateral upper lung lobar multiple thick atelectatic/fibrotic bands.
- No hilar or mediastinal lymph nodes.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.

OPINION:

- Bilateral moderate pleural effusion with underlying consolidation collapse.
- Bilateral upper lung lobes extensive ground glassing mounting to pneumonic consolidations with mild bronchiectatic changes.
- Bilateral upper lung lobar multiple thick atelectatic/fibrotic bands.

Dr. Mai Shaaban MD



Nihad Ali Mohammed

2026-06-01

Prof.Dr. no doctor

Gonstave Roussey International Egypt

Patient's Name:

Date:

Referred by:

Hospital:

NON-CONTRAST MULTI-SLICE CT SCAN OF THE PARA NASAL SINUSES IN THE CORONAL PLANE WITH SAGITTAL RECONSTRUCTION:

Findings:

- Mucosal thickening seen involving both maxillary antra with obliteration of the osteometal complex on both sides.
- Mucosal thickening is also seen partially opacifying the ethmoidal air cells on both sides.
- Mucosal thickening of both sphenoidal and left frontal sinuses.
- Clear other scanned paranasal sinuses with intact bony boundaries.
- No air/fluid levels are seen.
- The nasal septum is not significantly deviated.
- Clear nasopharynx.

OPINION:

- Findings are those of bilateral maxillary and ethmoidal chronic sinusitis.
- Mucosal thickening of both sphenoidal and left frontal sinuses.

Dr. Mai Shaaban MD

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Shahid

Prof. Dr. Shady Elia Anis

Elia Amir

Prof. Dr. Elia Anis Ishak

Block, findings are compatible with large B-cell lymphoma of post germinal center origin (activated B-cell type), triple expressor.

Conclusion:

Neoplastic lymphoid cells are POSITIVE for CD 20.
Neoplastic lymphoid cells are NEGATIVE for CD 3.
90 % of neoplastic lymphoid are POSITIVE for Ki67.
Neoplastic lymphoid cells are NEGATIVE for CD10.
40% of neoplastic lymphoid cells are POSITIVE for C-MYC.
Neoplastic lymphoid cells are POSITIVE for Bcl 2.
Neoplastic lymphoid cells are POSITIVE for Bcl 6.
Neoplastic lymphoid cells are POSITIVE for MUM 1.

Results:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain.

Technique:

IMMUNOHISTOCHEMISTRY REPORT

Receiving Date : 2026-05-08
Patient ID : 771992

Nature of Specimen : Block (Endoscopic biopsy, labelled as 1053620)

Clinical Diagnosis : Large malignant ulcer invading gastric antral mucosa and extending to duodenal bulb

Referred By Prof. Dr. : **عبدالله بن عبدالمطلب**

Age : 65 Y : 5 M : 7 D
Gender : Male

Conf. Code : 0600

Patient Name : _____

Analysis ID : 1055/69

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Elia Anis Ishak

DR.

M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Shady Elia Anis

Dr.



علاوة رئيس - بلاك أ - ميدان رئيس - اللور السلس و السلس
تليفون : ١٢١٩٤٤١١٥ - ٠١٢١٧٠٠٥١٧

1/1

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Prof. Dr. Elia Anis Ishak

Prof. Dr. Shady Elia Anis

Diagnosis :
Gastric ulcer, Endoscopic biopsy, MALIGNANT ROUND CELL TUMOR, LIKELY LYMPHOMATOUS.
- CASE FOR IMMUNOPHENOTYPING FOR CONFIRMATION AND SUBCLASSIFICATION.

Sections examined from the specimen revealed pieces of gastric mucosa with focal ulceration overlying dense diffuse infiltration by large and intermediate sized round cells with large nuclei and prominent nucleoli with moderate mitotic activity. There are small curved bacilli related to surface epithelium.

Gross :
Multiple tan tissue pieces, collectively measured 0.7x0.7 cm, totally submitted.

PATHOLOGY REPORT

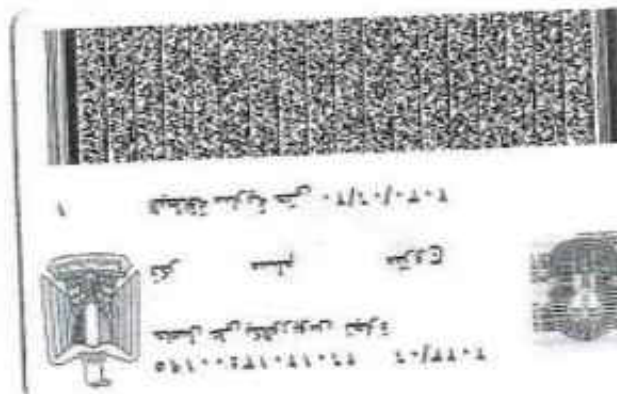
Patient Name : علي محمد منصور
Age : 65 Y : 5 M : 1 D
Gender : Male
Referred By Prof. Dr. : علي عيسى
Clinical Diagnosis : Large malignant ulcer invading gastric antral mucosa and extending to duodenal bulb
Nature of Specimen : Endoscopic biopsy
Receiving Date : 2026-05-02
Patient ID : 771992

Analysis ID : 1053620

Conf. Code : 1990

Dr. Shady Elia Anis
M.Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Dr. Elia Anis Ishak
M.Sc. Ph.D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology





S C A N

Name: Nehad Ali Mohamed Mansour
Referred by: Prof. Kerollos Atef Gaber

ID: 5154475
Date: 04/05/2026

¹⁸F-FDG PET/CT examination

Clinical Indication:

- Patient with history of surgically treated left breast cancer followed by chemoradiotherapy ended in 2019.
- Currently presented with suspected gastric cancer.

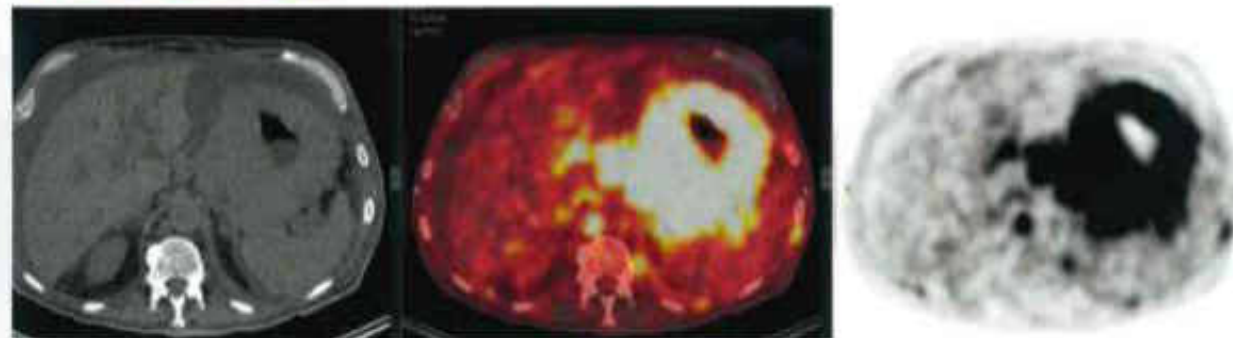
Administered activity: 244.2 MBq. ¹⁸F-FDG B.G.L. : 75 mg/dL

Examination protocol:

- A Whole body PET/CT study was obtained from skull vertex to the mid thigh using a low dose non contrast CT for attenuation correction
- Non-Contrast multislice CT examination of the same region.

In reference to the previous FDG PET/CT report dated 02/03/2022 (no DICOM images were available at time of interpretation)

I. PET/CT Findings:



- **Stomach:** increased FDG uptake corresponding to mural thickening involving the gastric fundus, body and antrum, SUVmax 11.8 as measured over 5.5 cm thickness in the body.
- **Lymph nodes:** increased FDG uptake by:
 - **Cervical:** bilateral levels II, III and right IV nodes, SUVmax 6.8 as measured over 1.8 cm left level II node.

الفاسكان

16171

AMG

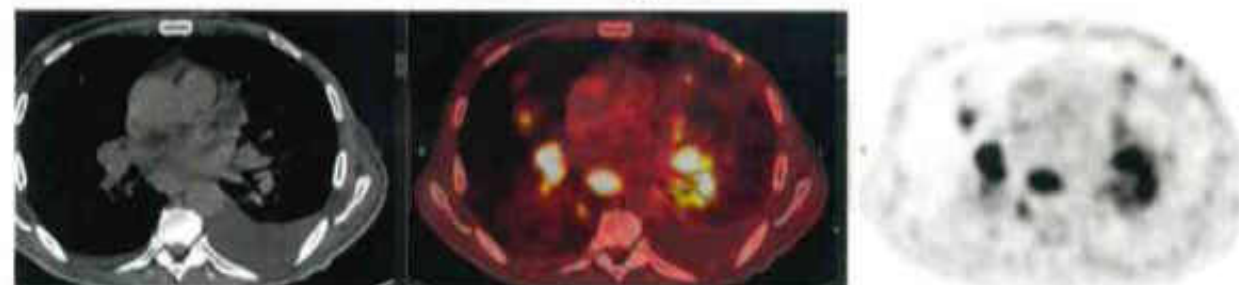
A MEMBER OF ALFA MEDICAL GROUP



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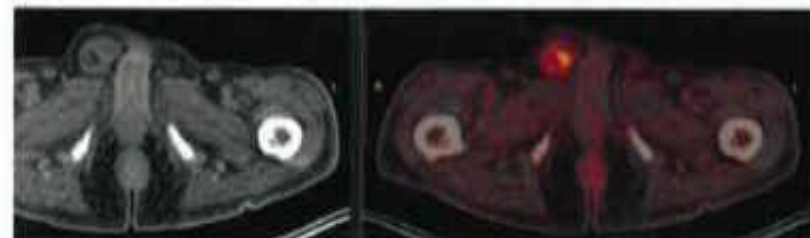


- **Mediastinal:** bilateral internal mammary, superior mediastinal, pre-vascular, pre-tracheal, bilateral hilar, aortopulmonary, sub-carinal, bilateral retrocrural and epiphrenic nodes, SUVmax 8.5 as measured over 2.7x 2 cm right hilar LN.



- **Abdomino-pelvic:** perigastric, celiac, porta-hepatis, porto-caval, aorto-caval, left para-aortic, mesenteric, bilateral common and left external iliac nodes, SUVmax 14 as measured over 3.9x 2.3 cm porto-caval LN.

(Newly developed in reference to the previous PET/CT report)



- **Peritoneum:**

الفاسكان

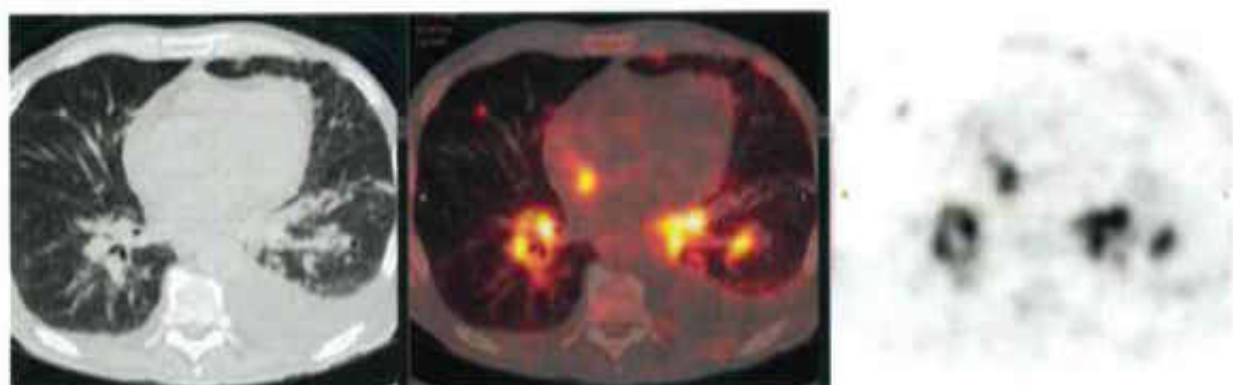
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A MEMBER OF ALFA MEDICAL GROUP

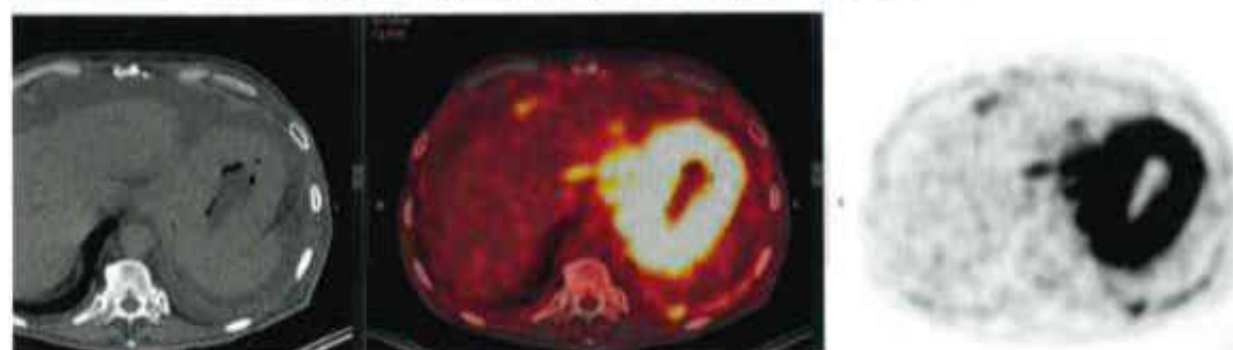
- increased FDG uptake corresponding to sheet like thickening seen perigastric, perihepatic (SUVmax 3.3), perisplenic, underneath anterior abdominal wall, para-colic, along pelvic reflections and along right inguinal hernia, the latter with SUVmax 4.7. (newly developed in reference to the previous PET/CT report).

- Moderate ascites.



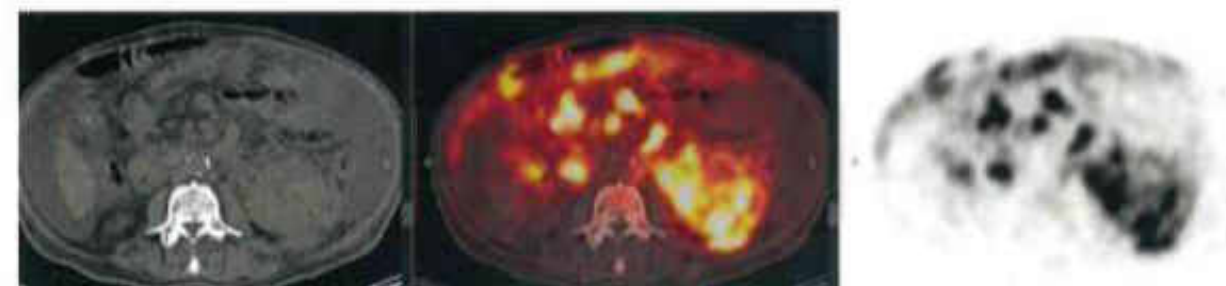
• Lungs:

- FDG avid bilateral multiple pulmonary nodules and nodular densities, SUVmax 2.5 as measured over 5 mm in the middle lobe. (newly developed in reference to the previous PET/CT report).
- Bilateral atelectatic changes and ground glass densities.



• Pleurae:

- Increased FDG uptake corresponding to mild sheet like thickening involving bilateral costal pleurae and along pleural recess, SUVmax 3.7 as measured over left posterior costal pleura. (newly developed in reference to the previous PET/CT report).
- Bilateral mild pleural effusion more on the left side associated with relaxation atelectasis.



- **Left kidney:** increased FDG uptake within ill-defined lesion infiltrating the whole parenchyma, SUVmax 7.9. (newly developed in reference to the previous PET/CT report).



- **Small bowel:** multiple areas of mural thickening are seen involving jejunal and ileal loops with increased FDG uptake, SUVmax 14.3 within ileal loop in the left iliac fossa.
- **Right kidney:** no FDG avid lesions.
- **Liver:** average sized with no FDG avid lesions. No dilated biliary radicles.



FDG uptake within bilateral nodular adrenal thickening,
0 on the right side, for follow up.

OW:

OG avid lesions.

enerative changes of the spine.

DG avid lesions.

ctomy bed and axillary dissection bed: no
sions.

st: no FDG avid lesions.

3 uptake within enlarged thyroid gland, *for
and sonographic correlation.*

CT Findings:

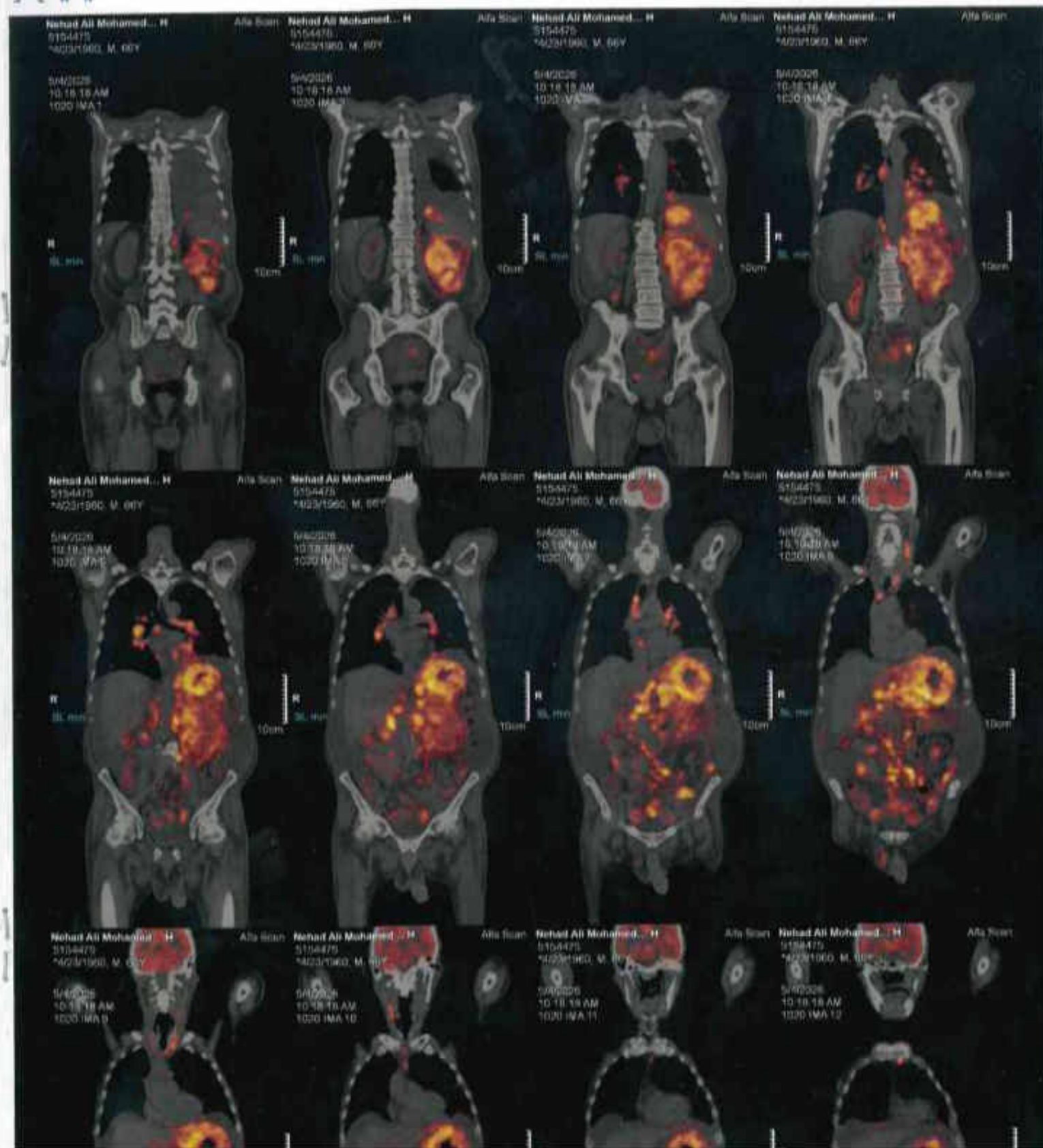
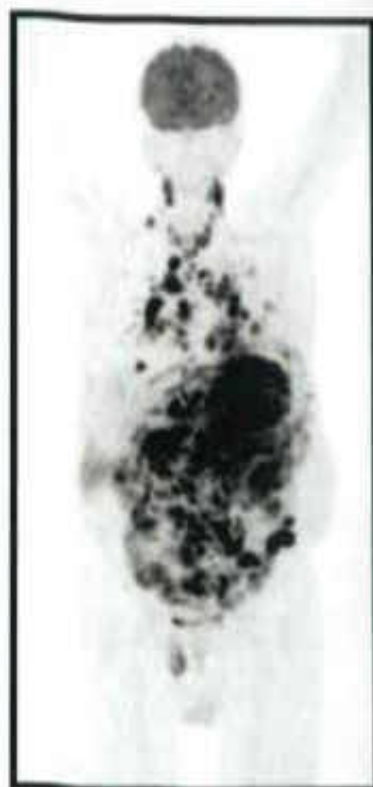
, pancreas, pelvic organs, aorta and IVC
markable.

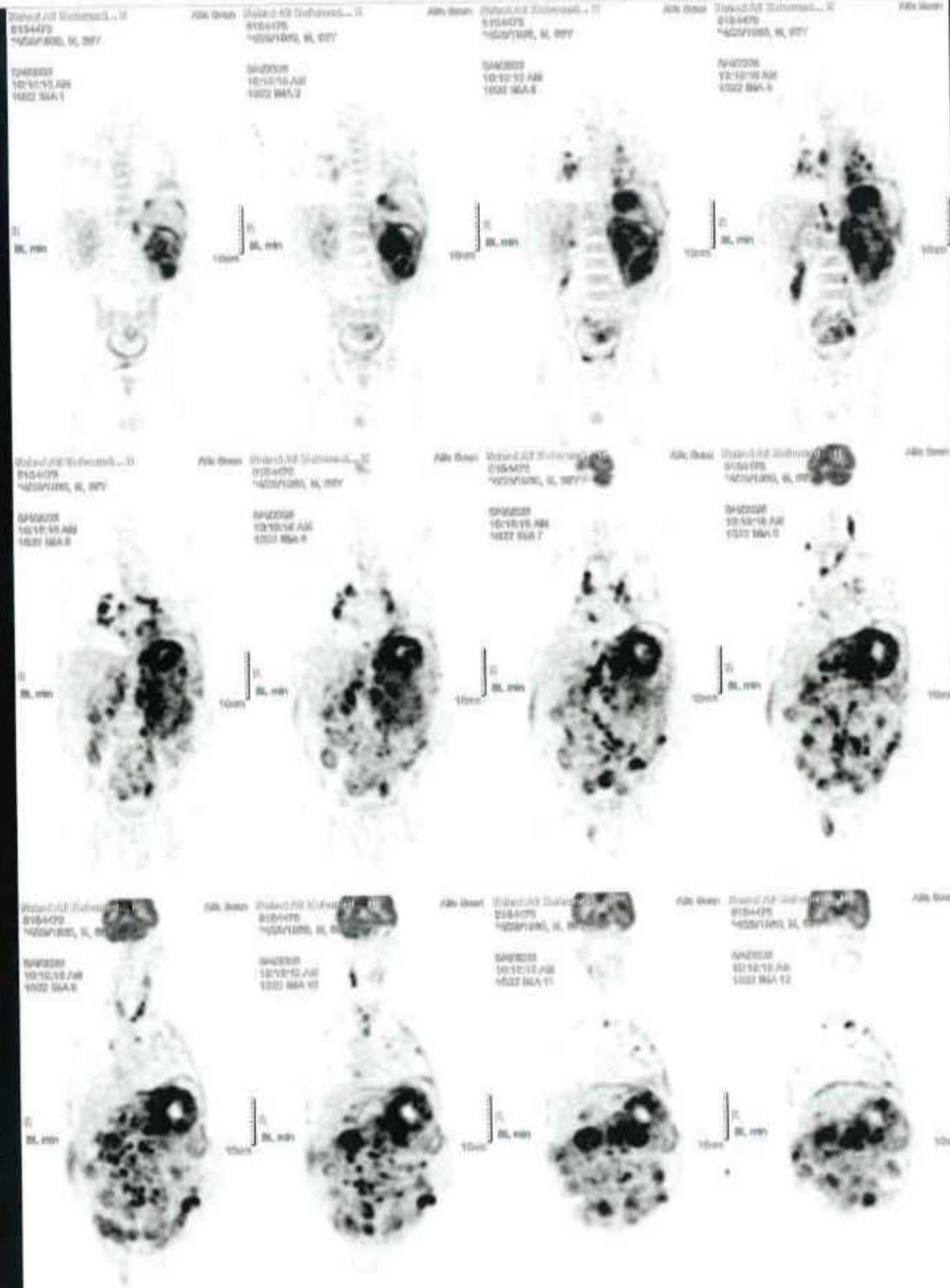
rdial effusion.

ily active gastric, lymph nodal, peritoneal, pulmonary,
all bowel and left renal neoplastic lesions as described.
is among diagnostic possibilities, for histopathology.

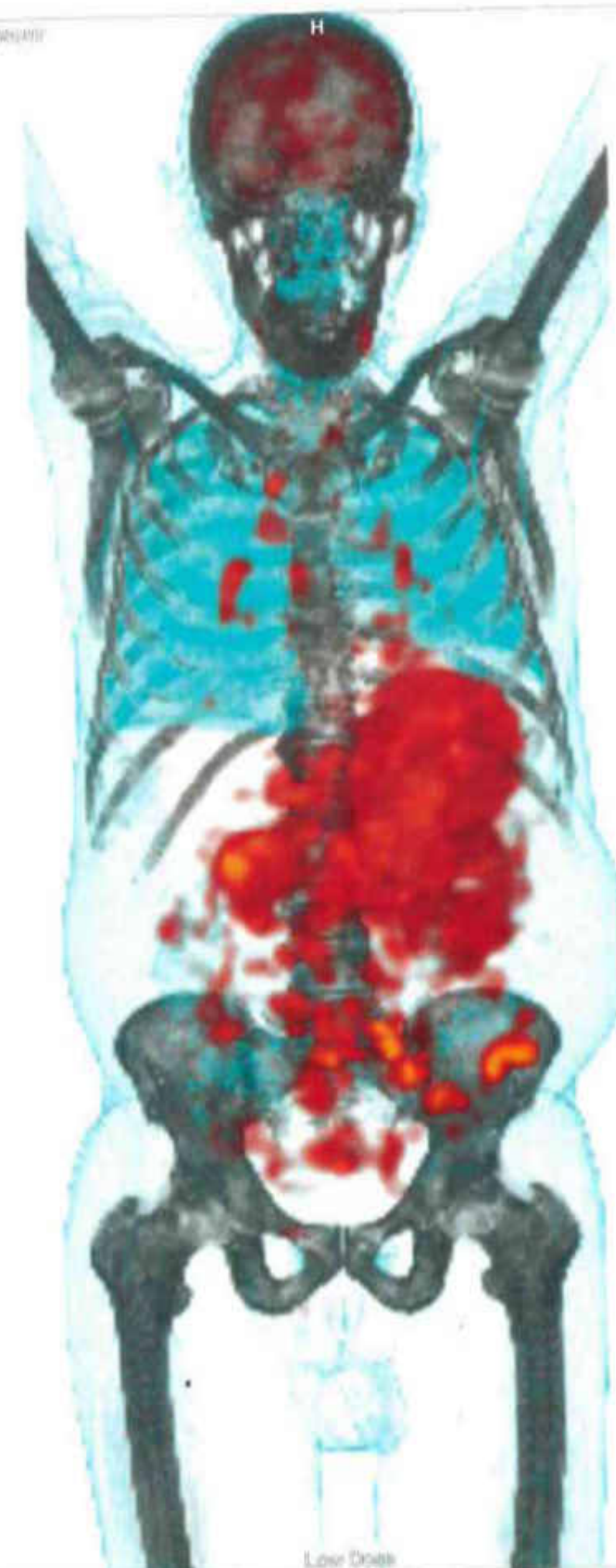
st
seny, MD

Nuclear Medicine Physicians
Dr. Sherif El-Refaei, MD, EBNM
Dr. Maha Khalil, MD





Abdel Fattah Mohamed, M
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10/10/19, M, 00Y
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Abdel Fattah Mohamed, M
01/04/75
10/10/19, M, 00Y

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إسم المريض ثلاثي: رضا رضا رضا
رقم الملف:

تقرير طبي

المعد:

تحية طيبة وبعد،،

إيماء الطالب المقدم من

بقسم: رقم طبي

تنتعرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول: تاريخ وساعة الخروج:

وبعد الكشف والفحوصات والمتابعة وجد أنه:

المريض يعاني من آلام بالصدر اليساري (DLCL)
ومبتدأه رجحاناً زرعياً وعلماً به كتاً، ١-٢

Anacoin
Atrialmga in
(CHOP) Endoxin
std
بواحي صريحه
١٣-١٤

مع ١٤-١٥ مع ألمة اليمنى
مع ١٤-١٥ مع ألمة اليمنى والتخارجة (ألمة غلب)

تحريراً في

التاريخ: ١٤٠١/٩/٢٠

الوقت:

الطبيب المعالج
الطبيب العام

يؤكد:

اسم المريض ثلاثي: زيار علي محمد
رقم الملف:

تقرير طبي

الميد:

تحية طيبة وبعد،،

إيماء للطلاب المقدم من

بقسم: رقم طبي:

نشرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول: تاريخ وساعة الخروج:

وبعد الكشف والفحوصات والمتابعة وجد انه:

المرضى ياتي من مستشفى بالسر المنيرة (Al BCL

CD20 +ve)

والمرضى يتكلم لا يستطيع ان يمشي بها الى السر (Rutimach) ما قبل

بفترة 7-10 يوم. حذرت كل من الاستماع لدرجة حرارته

وذلك لظهور استجابة جيدة له بعد 3 ساعات

pet ct 12/9

(Marked Regression)

تحريراً في

التاريخ: ٢٠١٩/٩-٢٠

الوقت:

الطبيب المعالج

د. محمد علي

يؤكد: ..

Name : Nihad Ali Mohammed Mansour	Date : 12-09-2026 07:03
Branch : Nasr city	Medical No. : 459430
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التأمين الصحي

Follow-up Whole body (FDG) PET/CT Study:

Clinical history:

- 65-year-old male patient with a history of surgically managed left breast cancer, with recent biopsy-proven large B-cell lymphoma from large gastric ulcerative lesion.
- He received chemotherapy till 30/08/2026
- PET/CT is requested for status evaluation and therapy monitoring.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is **< 2.5.**

PET/CT findings:

Considering the previous PET/CT report dated: 04/05/2026.

Head and neck:

- Metabolic and morphologic regression of the prior reported multiple enlarged bilateral upper/lower deep cervical currently few, reaching (1 cm compared to 1.8 cm) and eliciting SUVmax up to 2.6 compared to 6.8.
- No other pathologically enlarged or tracer-avid cervical or supraclavicular LN.s.
- Bilateral symmetrical tracer distribution is seen within the brain parenchyma with no abnormal metabolic activity or definite SOLs could be detected.
- Normal appearance of soft tissues and glandular structures of head & neck.

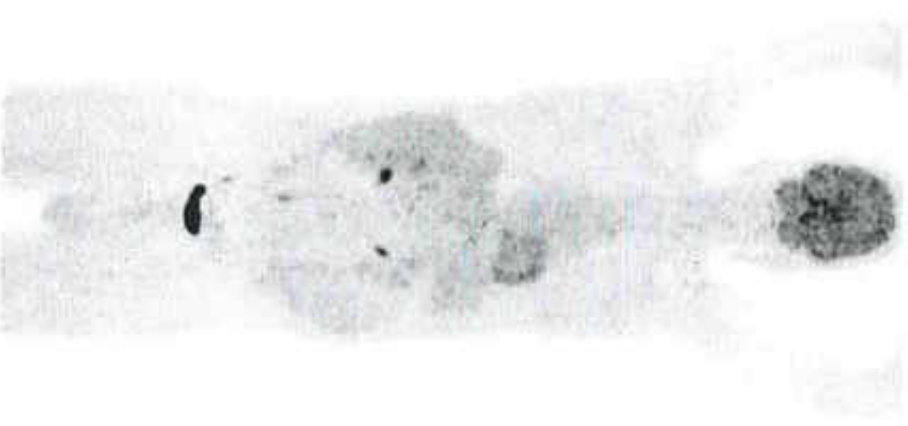
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Chest:

- Clear left mastectomy bed.
- Normal right breast appearance.
- Almost resolution of the prior reported mediastinal and bilateral hilar lymph nodes.
- No current definite lung nodules or parenchymal infiltrates.
- No current pleural thickening, masses or effusion.

Abdomen and Pelvis:

- Metabolic and morphologic regression of the prior reported FDG-avid diffuse gastric mural thickening currently up to 3.2 compared to 5.5 cm in max thickness and eliciting SUVmax up to 2.6 compared to 11.8.
- Metabolic regression of the prior reported left renal ill-defined infiltrating mass currently eliciting SUVmax up to 2.9 compared to 7.9.
- Almost resolution of the prior reported abdomino-pelvic lymph nodes currently sub-cm with no significant metabolic activity.
- Metabolic remission of the prior reported bilateral nodular adrenal thickening.
- Almost resolution of the prior reported omento-peritoneal nodular thickening.
- Average-sized liver showing homogenous CT-texture and tracer uptake pattern with no definite focal lesions (on non-contrast CT basis), tracer-avid foci or biliary radicles' dilatation could be detected.
- Unremarkable spleen, right kidney, pancreas and pelvic organs.
- No current evidence of ascites, free or encysted abdomino-pelvic fluid collections.



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Musculoskeletal & BM:

- No any pathological sclerotic/lytic bony lesions or abnormal tracer-avid foci could be detected over the surveyed skeleton.

Impression:

Follow-up whole-body (FDG) PET/CT study shows:

- Maintained Clear left mastectomy bed.
- Metabolic and morphologic regression of the prior reported FDG-avid diffuse gastric mural thickening.
- Metabolic and morphologic regression of the prior reported multiple enlarged cervical lymph nodes, curranty coping with Deauville score III.
- Metabolic regression of the prior reported left renal ill-defined infiltrating mass.
- Almost resolution of the prior reported mediastinal bilateral hilar, abdominal, pelvic lymph nodes as well as peritoneal, pulmonary, pleural and small bowel infiltrates.
- No newly developed nodal or extra nodal metabolically active lesions.

Best regards,

Dr. Youssef Ragab Gad-Allah, MSc.

Dr. Alaa Nagi, MD.
Nuclear Medicine



